جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ياسمين زهران جابر محمد
المحافظة : الجيزه **الحي / المدينة :** العمرانيه **العنوان :** ممم
رقم البطاقة : 27905150105123 **رقم التليفون :** 01118503806 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 15-05-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ياسمين زهران جابر محمد
National ID: 27905150105123

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

USTAVE OUSSY
NATIONAL

Patient Name	: الأستاذه / ياسمين زهران جابر	Request No.	: 13005450
Patient No.	: 27905150105123	Request Date	: 14/03/2026
Path No.	: 710/26	Verification Date	: 23/03/2026
Sex	: Female	Financial Category	: جوستاف رومي - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of the specimen: Left breast conservative surgery & Axillary lymph nodes dissection.

Gross Examination:
• Left breast conservative surgery specimen measured 10x07x04 cm, with covering skin ellipse measured 5.5x1.5 cm.
• Serialling showed a wire guided tumor bed site measured 2.3x2x1 cm, located 0.5 cm from the least lateral margin.
• Axilla: Axillary fat measured 8x7x2 cm, entangling 11 lymph nodes, the largest was 2x1.5x1 cm.
- 11 Blocks:
1- Medial margin., 2- Lateral margin., 3- Superior matgin., 4- Inferior margin., 5- Deep margin., 6:7- TBS., 8&11- ALNs.
- Gross was done by; Dr. Heba Hassan.

* Frozen (Intra-operative) Results:
- Free surgical margins.

Microscopic examination:
• Histological type: Residual viable invasive duct carcinoma with mucinous differentiation (Cellularity 2%) (Actual tumor size 0.4x0.4 cm).
• Grade 2 (differentiation 2, pleomorphism 2, mitosis 2).
• Ductal carcinoma in situ: Absent.
• Angioinvasion: Present.
• Perineural invasion: Absent.
• Surgical margins: Free.
• Axillary lymph nodes: Positive viable metastatic deposit involving one out of eleven lymph nodes (1/11) (The metastatic focus measuring 1.5 cm in greatest dimension, with extra-nodal

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE OUSSY
INATIONAL

Patient Name	: الأستاذه / ياسمين زهران جابر	Request No.	: 13005450
Patient No.	: 27905150105123	Request Date	: 14/03/2026
Path No.	: 710/26	Verification Date	: 23/03/2026
Sex	: Female	Financial Category	: جومتاف روسي - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

extension).
• Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:
Left breast cancer (Post-NACT), Left breast conservative surgery & Axillary lymph nodes dissection:
• Residual viable invasive duct carcinoma with focal mucinous differentiation, Grade 2, Cellularity (2%), Actual tumor size (0.4x0.4 cm).
• Ductal carcinoma in situ: Absent.
• Surgical margins: Free.
• Axillary lymph nodes: Positive viable metastatic deposit involving one out of eleven lymph nodes (1/11) (The metastatic focus measuring 1.5 cm in greatest dimension, with extra-nodal extension).
• Pathologic partial response to therapy (PPR).
• Stage: ypT1a, ypN1a.
• RCB: 2.797, RCB class-II.

Signature

Prof. Dr. Gamal Abd-El-Aty.
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: ياسمين زهران جابر محمد **MR. :** 668	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has a left breast cancer staged as CT2 N1 M0, with ER 8/8, PR 8/8, and HER2 amplified. The treatment plan included neoadjuvant Taxotere and Carboplatin with surgery pending, followed by continuation with PHESGO beginning at cycle 2.

Neoadjuvant TCHP was completed, and the patient was scheduled to receive PHESGO for the left breast, with PHESGO planned for cycles 3 and 4 and pharmacy coordination pending prior to surgery.

PHESGO at the dose of 600 mg/600 mg was intended for cycle 5 but was postponed due to drug availability.

The patient's breast cancer management was planned to start with PORTH and adjunctive KADCYLA on the same day, initiating the first month of Femara, with Femara planned for a total of three months.

On the day of cycle 1 of KADCYLA, laboratory results were reviewed, Femara was continued, and supportive treatment was provided; an echocardiogram performed on 12-Mar-2026 showed an ejection fraction of 68 %.

Approval for KADCYLA had not yet been obtained, and the indicated procedure was deemed not applicable to the patient; only two doses of PHESGO were to be given before surgery according to the new MDT recommendations.

Approval for KADCYLA, Femara, and Zoladex was obtained, and laboratory results for cycle 1 were accepted. The patient has completed cycle 1 of the regimen.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **Ali Adel**

يعتمد ,



عاطل

DMS

كاسيل

National ID · Front
[[DOCTYPE:ID_FRONT]]



National ID · Back
[[DOCTYPE:ID_BACK]]

مبـــادرة رئيس الجمهوريـة
لدعـم صحة المرأة المصرية

04/07/2026

MDT FORM

Name:	Yasmin Zahran Jaber Mohamed	**MRN:**	668
Age:	47 Y	**ID:**	27905150105123
Menopausal Sta		**Phone:**	01112439190
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	NEGATIVE
CO Morbidity:	DM	**Reason Of MDT:**	POST OP

Clinical Data:

BX: IDC 2

ER: 8/8 **PR:** 8/8 **HER2:** AMPLIFIE **KI67:** 25%

Sonomammography:

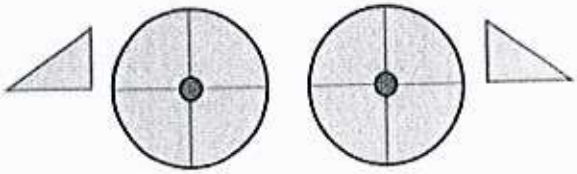
Mets Workup:

History:
PT UNDERWENT, LT BCS + ALND ,POSTNACT, WITH FREE SURGICAL MARGINS
AND 1/11 WITH EXTRANODAL EXTENSION
PATHOLOGIC PARTIAL RESPONSE TO THERAPY
STAGE T1A, N1A

Recommendations:

MDT Outcome & Plan
for adj Kadcyla
PORT
femara 7 YEARS

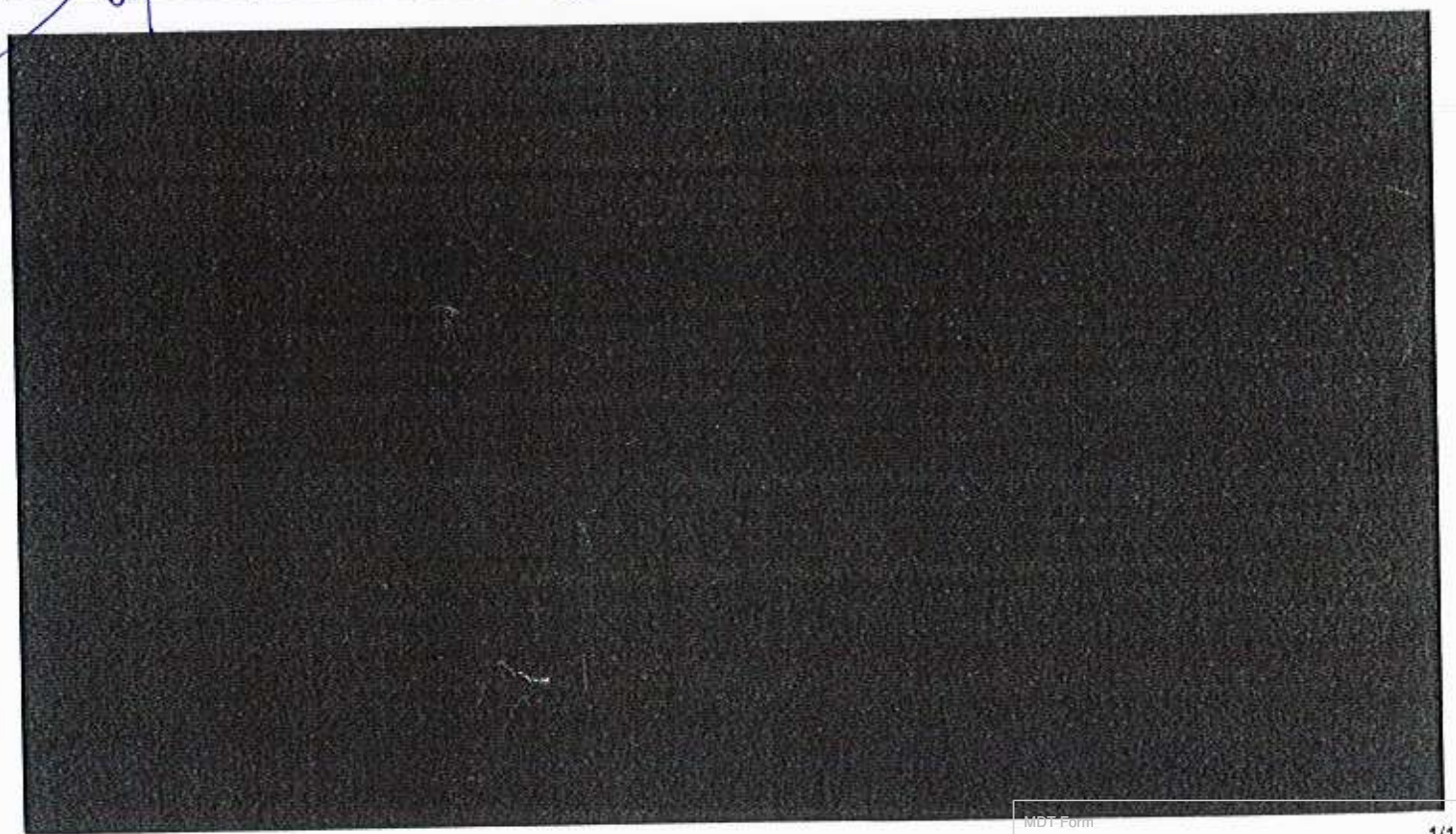
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TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
حنان عزت					



جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

New **MDT FORM** 2/9/2025 G

Name	ياسين زكريا بابر	**MRN**	٢٢٢٦ | ٥٥٥
		ID	٢٨٠٩٢٢٠١١١
Age	46 y	**Phone**	٠١٠٦٢٦٢٩٥٠٤
Menopausal status	menopausal	**Marital status**	married
NRS SCORE		**Family history**	-FH
BMI		**REASON OF MDT**	New
CO morbidity	NAD	٢٧٩٠١٠١٠٠١٠١٢٣	

Lt IDC G II

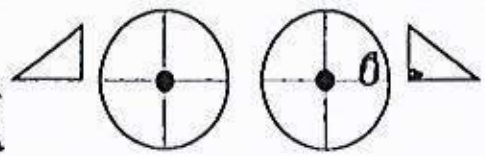
Clinical Data

BX: Amplified

ER:	PR:	HER 2:	Ki67:
8	8	++2 SISH GT	25%

Sonomammography: Lt UIQ 11-12 o'clock 3 x 3.5 cm deep to skin 1.5 mm L.N ileo x 1

R MDT L



Mets Workup: CT CAP ⇒ free

TNM 2 1 0

Recommendations: For possible clip insertion & neoadj chemo.
د/ ياسر

Retroareolar micro Calcification. Reviewed By Radiologist
No need for CESM

MDT OUTCOME & PLAN
1. Clip insert
2. Start 6 cycles TCHP
3. BCS + ALNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ هالة	Dr. Hoda 2/9/2025	د/ ياسر			

MDT Form
[[DOCTYPE:MDT_FORM]]

Name	:الاستاذه / يسمين زهران جابر محمد	Request No.	:13000783
Civil No.	:27905150105123	Request Date	:07/08/2025
Patient No.	:SR-1086/25	Verification Date	:11/08/2025
Sex	:Female	Financial Category	:جوستاف روسي - فئة الدولة
Age	:46 Y	Department	:Out Patient
Referreing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Diagnosis:
Left breast mass & left axillary lymph node, Referred blocks & slides (Tru-cut biopsies), Both (1 & 2):
- Invasive duct carcinoma, Grade 2.
- 2% Intermediate grade DCIS (Solid pattern).

IPT:
- ER: Both (1 & 2): Positive (score 8/8).
- PR: (1) Positive (score 5/8).
  (2) Positive (score 8/8).
- Her2: Both (1 & 2): Equivocal (score 2+).
- SISH: Amplified, Group I.
(Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (4.3)).
- Ki67%: Both (1 & 2): 25%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
he invasive tumor cells or complete and circumferential membrane staining that is intense

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

الأستاذة / ياسمين زهران جابر محمد

27905150105123
SR-1086/25
Female
46 Y

Referring Doc :

Request No. : 13000783
Request Date : 07/06/2025
Verification Date : 11/06/2025
Financial Category : عيادات خارجي - علاج على نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Yasmin Zahran Gaber Mohamed	Date : 19/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID: 2238/2025	Pat. ID: 111011

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Left breast mass lesion is seen measuring 36 x 31 mm associated with mild skin thickening for sonomammography.
- Left axillary pathologically enlarged lymph node measuring 35 mm.
- Tiny right upper lobe calcified pulmonary nodule.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric para-aortic and bilateral iliac lymph nodes.
- No ascites.
- Bone survey revealed no lytic or sclerotic bone lesions.
- For PET-CT assessment.

Dr. Sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

MRN:	10125267708
Patient Name:	YASMEN ZAHRAN GABER MOHAMMED
D.O.B:	15/05/1979
IC No.:	27905150105123
Sex:	Female
Location:	Surgical Oncology

SURGICAL PATHOLOGY

Slide Review (5 slides)

Specimen Id	2507230027	**Collection Date**	23/07/2025 09:06 AM
Accession No	SUR25-4430	**Received Date**	23/07/2025 09:06 AM
Sample Type	Slide/ Block	**Doctor**	Yassmin Mohamed Hassan Riad

Clinical Information

Left breast and left axillary lymph nodes, US guided core biopsy, for review.

Gross Picture

Received two paraffin blocks, two H&E stained slides (one labelled as 1 (breast) and other 2 (lymph node)) and four immunostained slides. All material is coded 25018105.

Microscopic Picture

Referred material show a case of invasive duct carcinoma, grade 2. No DCIS component was detected. Stroma is desmoplastic with mild inflammatory cell reaction. With axillary lymph node shows metastatic deposits.

Review of the referred immunostained slides revealed :
ER: Positive, strong nuclear reaction in 80% of tumor cells (8/8), both sites.
PR: Positive, strong nuclear reaction in 70 % of tumor cells (8/8), both sites.
Her-2 /neu: equivocal (score +2), both sites.
Ki-67 labelling index : 25 %, , both sites.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, grade 2 with axillary lymph node deposits.

Laterality

Left

Specimen type:

Guided core biopsy

Location

Breast and Axillary lymph node.

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

2

Nuclear grade (1-3):

2

Histological grade (1-3):

2

Diagnosis

Left breast mass and left axillary lymph node, US guided core biopsy, revision:
Invasive duct carcinoma, grade 2 with metastatic axillary lymph node.

Review of the referred immunostained slides revealed :

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

MRN:	10125267708
Patient Name:	YASMEN ZAHRAN GABER MOHAMMED
D.O.B:	15/05/1979
IC No.:	27905150105123
Sex:	Female
Location:	External Clinic

MOLECULAR GENETICS

Sish

Specimen Id	2507260082	**Collection Date**	26/07/2025 09:44 AM
Accession No	SISH25-296	**Received Date**	26/07/2025 09:44 AM
Sample Type	Tissue	**Doctor**	Abd ALLAH Mohamed Gamil

Sish Report

pathology number

SUR25-4430

Clinical Data

Left Breast Invasive duct carcinoma, grade 2 with axillary lymph node deposits

Gross Picture :

Referred blocks submitted for HER 2 detection by silver in situ hybridization.

System Used :

Ventana BenchMark ULTRA autostainer.

Probe:

Ventana Inform HER2 Dual ISH DNA Probe Cocktail

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations,2018 Twenty nuclei (each containing red (Chr17) and black (HER2) signals) should be enumerated. The final results for the HER2 status are reported based on the ratio formed by dividing the sum of HER2 signals for all 20 nuclei divided by the sum of Chromosome 17 signals for all 20 nuclei. The amplification status is defined as Amplified if the HER2/Chromosome 17 ratio >/=2.0 and the average Her2 gene copy number is >/=4.0. It is Non-Amplified if the HER2/Chromosome 17 ratio <2.0 with the Her2 gene copy number is <4.0.

If the HER2/Chr17 ratio is <2 and the Her2 gene copy number is between 4.0 and 6.0, or, HER2/Chr17 ratio is >/=2 and the Her2 gene copy number is <4, or HER2/Chr17 ratio is <2 and the Her2 gene copy number is >/= 6.0, an additional work should be done.

(Laterality) breast Cancer:

Left

Site 1

Laterality #1

Left Breast.

Diagnosis #1

Amplified: Her2 copy No: 4.2, Her2/CEP ratio: 2.9.

Final Result #1

Amplified

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

BAHEYA

PATIENT NAME: YASMEN ZAHRAN GABER MOHAMMED
GENDER: Female
DATE : 21-Jul-2025 09:49 AM

PATIENT ID: 10125267708
AGE: 46 y
ORDER: 3481033,3481034

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Pathological proven malignant left breast mass outside Baheya, IDC.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).

Left breast:

- UIQ peripherally located irregular mass with high density measuring 42x48mm, away from the nipple by 83mm measured on CC view. Few overlying amorphous microcalcifications noted.

Complementary ultrasound:

- 11-12 o'clock malignant looking mass measuring 31x34x45mm, deep to skin by 1.5mm. Mild vascularity on color Doppler. **Multiple prominent hypoechoic ducts** noted towards the nipple, yet with *no* vascularity detected on color Doppler.
- Few faint widely spaced retro-areolar microcalcifications noted.
- Few pathological **axillary level I** lymph nodes (about 3) are noted. The largest measuring 24mm in short axis with muffled eccentric hilum.
- No pathological infra or supraclavicular lymph nodes.

Right breast:

- No spiculated mass lesions or grouped microcalcifications.
- Complementary Ultrasound which showed no solid mass lesions.
- Multiple scattered simple cysts.
- Prominent retro-areolar ducts with inspissated secretions.
- 7 o'clock zone C area of hypoechoic area of confluence of ducts measuring 11x6mm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Left UIQ pathological proven malignant mass as described (BIRADS 6) associated with few pathological axillary left level I lymph nodes for histopathological assessment.
- Right 7 o'clock area of confluence of ducts (BIRADS 3) for sonographic follow up.

RECOMMENDATION:

Referral to clinic

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY INTERNATIONAL

ECHO REPORT

NAME : ياسمين زهراي بناير DATE: 3 / 12/2025

ID NUMBER : / SEX : AGE: 46

EF	68%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.7	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVESD	2.3	2-4cm	LA	3.8	2-4cm
FS	30%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GI

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve trivial MR

Aortic Valve N

Tricuspid Valve N

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.5

Intracardiac Masses and Effusion

DR.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]